



CLOVATE CREAM

Description: White colour cream.

Content

Clobetasol 17 propionate 0.05% w/w

Preservatives: methylparabens 0.1%w/w, propylparabens 0.01%w/w

Indications

For psoriasis, recalcitrant eczemas, lichen planus, discoid lupus erythematosus.

Pharmacology

Clobetasol propionate an adrenocorticoid diffuse across cell membranes and complex with specific cytoplasmic receptors. These complexes then enter the cell nucleus, bind to DNA (chromatin) and stimulate transcription of messenger RNA (mRNA) and subsequent protein synthesis of various enzymes thought to be ultimately responsible for anti-inflammatory effect of topical application of adrenocorticoids. The vehicle of a topical adrenocorticoid formulation may also contribute to the therapeutic effect by providing an emollient action, a drying action, or increased transcutaneous absorption of the adrenocorticoid. Absorbed systemically. The extent of absorption of a topical dosage form may be influenced by the vehicle used in a specific formulation. Occlusion, use over extensive areas, a prolonged use, increases absorption of topical dosage forms. Absorption increases with increased potency. Metabolism is mostly in the skin, fluorinated compounds are slowly metabolised in skin and thus tend to be systemically absorbed to a greater extent. Clobetasol is excreted in the urine.

Adverse effects

Side effects may be more pronounced with the use of an occlusive dressing, also they may be more severe with fluorinated preparations. If a local infection develops at the site of application, discontinue occlusive dressing (if used) and institute appropriate antimicrobial therapy. Because of some systemic absorption of the topical adrenocorticoids, systemic effects including Cushing's syndrome, hyperglycemia and glucosuria, may occur with prolonged use.

Precautions

Pregnancy or Reproduction - Risk benefits must be considered since studies in animals have shown that topical adrenocorticoids when used in large amounts or for prolonged periods of time are systemically absorbed and may cause fetal abnormalities.

Breast feeding - It is not known whether topical adrenocorticoids are excreted in breast milk.

Paediatrics - Paediatric therapy continuing for longer than 2 weeks and consisting of doses in excess of 1 daily application (with intermediate or high potency corticoids) or 2 daily applications (with low potency corticoids) should be evaluated carefully by the physician.

Risk benefit should be considered when the following medical problems exist: infection or ulceration at site, herpetic lesions, diabetes and tuberculosis.

Drug interaction: Concurrent administration of barbiturates, carbamazepine, phenytoin, primidone or rifampicin may enhance the metabolism and reduce the effects of corticosteroid. Concurrent administration of corticosteroids with potassium depleting diuretics, such as thiazides and furosemide, may causes excessive potassium loss. There may an increased incidence of gastrointestinal bleeding and ulcerations when given with non-steroidal antiinflammatory agents.

Response to anticoagulants may be altered by corticosteroids and requirements of antidiabetics and antihypertensives may be increased. Corticosteroids may decreased serum concentrations of salicylates and may decrease the effect of antimuscarinic in myasthenia gravis.

Contraindications

Topical applications of corticosteroids should not be made with an occlusive dressing to large areas of body because of increased risk of systemic toxicity and should not, in general, be used in the presence of infection (bacterial or fungal) and primary cutaneous viral infections (e.g. herpes simplex and chickenpox), particularly of the eye. They should not be used for the treatment of rosacea and acne vulgaris, and should not be used indiscriminately for pruritis (e.g. perianal and genital pruritis). Corticosteroids should not be applied to ulcers of the legs and long term topical use is best avoided, especially in children. It should also be contraindicated in patients hypersensitive to the preparation and patients with peri-oral dermatitis.

Dosage and administration

Usual adult dose : Apply to the skin two times a day. Max: up to 50 grams per week.
The duration of therapy should be limited to 14 days.

Usual paediatric dose : Children below 12 years - use is not recommended.

Storage : Keep container tightly closed. Store in a cool, dry place below 30°C.
Protect from light. Keep away from children.

Presentation : 5gm/tube, 15gm/tube, 100gm/jar, 500gm/jar (EXPORT)

Shelf life : 3 years

Manufactured & : PRIME PHARMACEUTICAL SDN. BHD.

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